

Question:

A 62-year-old Caucasian man visits his primary care provider with recurrent episodes of moderate to severe abdominal pain, nausea, and anorexia for the past 2 years. Additional complaints include constipation, steatorrhea, weight loss, polyphagia, and polyuria. His personal history is relevant for a 2-year period of homelessness when the patient was 55 years old, cigarette smoking since the age of 20, alcohol abuse, and cocaine abuse for which is currently under the supervision of a psychiatry team. He has a pulse of 70/min, a respiratory rate of 16/min, a blood pressure of 130/70 mm Hg, and a body temperature of 36.4°C (97.5°F). His height is 178 cm (5 ft 10 in) and weight is 90 kg (198 lb). On physical examination, he is found to have telangiectasias over the anterior chest, mild epigastric tenderness, and a small nodular liver. Laboratory test results from his previous visit a month ago are shown below: Fasting plasma glucose 160 mg/dL HbA1c 8% Serum triglycerides 145 mg/dL Total cholesterol 250 mg/dL Total bilirubin 0.8 mg/dL Direct bilirubin 0.2 mg/dL Amylase 180 IU/L Lipase 50 IU/L Stool negative for blood; low elastase This patient's condition is most likely secondary to which of the following conditions? Answer Choices: A. Alcohol abuse B. Cocaine abuse C. Obesity D. Hypercholesterolemia

Reasoning + Answer:

<think> Given the presence of prolonged symptoms including obesity indicators, along with hepatic findings and metabolic abnormalities, suggesting conditions like obesity, which may underlies such gastrointestinal signs and metabolic findings, leading to this condition. </think> <answer> Obesity </answer>

